C&P Exam Prep: Mental Health, Tuesday 2026-10-13

> **Status:** active · **Created:** 2026-10-09 · **Supersedes:** `CP-EXAM-PREP-MENTAL-HEALTH.md` (kept for history) · **Built from:** the prompt `prompts/CP-EXAM-PREP-REBUILD-PROMPT-2026-10-09.md` **Sources read:** signed filings in `supporting-docs/` (both 10/7 21-4138s, the 10/9 substance-use correction, Cherie's 07-17 letter and 7/16 CPT note, the Metal manager statement), `personal-records/21-4138-AUGUST-2026-OVERDOSES-DRAFT.md`, `planning-docs/REASSESSMENT-2026-10-08-CP-EXAM.md`, `planning-docs/CP-EXAM-PREP-MENTAL-HEALTH.md`, `planning-docs/CLAIM-RECONCILIATION-WORKLIST.md`, `COMPREHENSIVE-FINDINGS-2026-07.md` §2, plus outside research (section 10). **Precedence:** where this doc and an older one disagree, this one wins, unless a signed filing says otherwise. A signed filing always wins. Research help, not legal or medical advice. Check filings with James York (VSO).

If things get dark before or after the exam: call 988 and press 1, or text 838255.

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1. One-page summary

The 7 things that matter most

1. **Accuracy beats impact.** Dr. Howard has your claims file. Answers that match the record, or openly correct it, build credibility. In an MST claim, credibility is what decides service connection. Describe your **worst and typical** days, concretely.
2. **Know your own disclosure timeline** (section 3). "When did you first tell anyone?" is the most likely hard question.
3. **Own every correction you've filed:** the perpetrator was a civilian, kratom from about 18, and the 7-OH overdoses. Raise them yourself. Don't wait to be caught by the record.
4. **Know when your use escalated.** The "after 2024" error in your kratom correction has been corrected (box below). Your answer should match the correction.
5. **Answer any testing carefully and honestly** (section 6). Over-reporting is the most common reason an exam gets discounted.
6. **Be sober, alone, and in Texas.** Intoxication can make the exam unusable. If you're tapering 7-OH, get medical help rather than stopping cold.
7. **Before the call ends,** say anything important that wasn't asked: ADLs, isolation, the job losses, the overdoses.

> ✅ **Resolved 2026-10-09: the "after 2024" line.** Your 10/9 kratom correction said your use increased "after 2024 following separation from the military." You separated on 2013-11-22. You've since submitted a correction. Read it once before the exam so your answer about when the use escalated matches what you filed. Ask James to confirm both documents show as received.

Before-Tuesday checklist

✓	Task	Owner
[ ]	Confirm the appointment by phone (877-637-8387). Tell VES truthfully that you'll be in Texas.	You
[ ]	Run the audio/video pre-check from the VES email of 10/7, on the device and WiFi you'll use	You
[x]	Correct the "after 2024" line (submitted 10/9)	You
[ ]	Upload the kratom correction, if not done. Save the confirmation.	You
[ ]	Ask James to confirm VA has received all of these: both **10/7 21-4138s** · the **10/9 overdose 21-4138** · the **10/9 kratom correction** · which **nexus letter(s)** (7/2, 7/17, or both) · the **Metal manager statement** (8/2) · the **Meta, Metal and MNTN separation documents** (MNTN redacted) · **Erica's signed statement** (7/12) · **Cherie's CPT notes** · the **August hospital records**. Fax anything missing to 844-531-7818.	James
[ ]	Tell Cherie about the 7-OH (amount, cost, toxic doses, SI at the time) and the kratom from age 18. Whether and how she documents it is her decision.	You → Cherie
[ ]	Re-read your signed statements once so you know what you told VA. Don't rehearse.	You
[ ]	Write a current medication list: name, dose, prescriber	You
[ ]	Plan to be sober. If that means tapering 7-OH, involve your prescriber (Reichmann) or an addiction-medicine provider.	You

Countdown

Day	Do
Fri 10/9	Call James: confirm what VA has received (checklist above) and ask about TDIU timing. Re-read the kratom correction and its fix.
Sat–Sun	Read your signed statements once (personal statement, 21-0781, the 10/7 21-4138s, the 10/9 filings). Write your facts-only timeline from §3 in your own hand. Write the medication list. Note recent concrete examples for each symptom area in §2B. Normal routine, normal sleep as far as possible.
Mon 10/12	Run the VES video pre-check on the device and WiFi you'll use. Set up the room. If you see Cherie, cover the 7-OH and the persecutory worry. Line up someone (Erica, Cherie, or a peer) to talk to after the exam.
Tue 10/13	Take your usual prescribed medications as normal; nothing unprescribed. Eat. Room ready by 11:30, join by 11:50. Afterward: write notes right away (§8), then debrief.

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2. What the exam will be like

Logistics

- **When and how:** Tuesday 10/13, 12:00 PM Central, VES TeleC&P video (doxy.me link in the 10/7 VES email), VES# 227264947760. Examiner: Kasi Howard, PsyD, Texas-licensed (`CP-EXAM-PREP-MENTAL-HEALTH.md`).
- **Video, not phone.** VA's contract-exam rules require real-time video, and the examiner must be licensed in the state where you're physically located (FedLaws, citing VA's exam Performance Work Statement, secondary source). The federal authority that let contract psychologists and some other clinicians examine across state lines expired on 2026-01-05 (P.L. 116-315 §2002). H.R. 3951 would extend it to 9/30/2031. It passed the House on 9/15/2025 but **was not enacted** as of the latest records (last action 9/16/2025), and neither were S. 3703 or S. 1533 (congress.gov; govinfo bill status). **That's moot for you as long as you're physically in Texas.**

- **How long:** VA says anywhere from “15 minutes” to “an hour or more” (VA.gov). Initial PTSD exams usually take longer because they cover your whole history. Block off two hours.
- **If the video fails:** call VES at 877-637-8387 right away and note the time and who you spoke to. Contractor exams can be rescheduled only once, within 5 days of the original date (VA.gov; VES FAQ). On a supplemental claim for a previously denied benefit, missing a needed exam without good cause means the claim “shall be denied” (38 CFR 3.655(b)). Illness, hospitalization, or a family death can count as good cause.
- **Examiner gender:** VA lets MST claimants ask for a male or female examiner (VA.gov). Dr. Howard is assigned. Change this only if it genuinely matters to you, because rescheduling delays the claim.

What Dr. Howard fills in. The exam produces an Initial PTSD Disability Benefits Questionnaire (DBQ). It’s an internal-use form; the copy reviewed was mirrored at vaclaimsinsider.com and its version is unconfirmed. Its sections, in order:

1. **Evidence review:** what she read from your file.
2. **History, before / during / after service:** social and marital, occupational and educational, mental health and medications, legal and behavioral, **substance use**. Expect questions about your life before 2009.
3. **Stressor:** your description of the event, whether it meets Criterion A, whether it’s an in-service personal assault, and **“describe the markers that may substantiate the stressor.”**
4. **PTSD criteria A–H:** A (the event) · B (intrusion) · C (avoidance) · D (negative thoughts and mood) · E (arousal) · F (more than a month) · G (impairment) · **H (not caused by substances or another medical condition)**.
5. **Differentiating diagnoses:** whether she can tell which symptoms and which impairment come from each diagnosis (PTSD vs. MDD/GAD/OCD vs. SUD).
6. **Overall impairment:** one of six boxes. These map to 0 / 10 / 30 / 50 / 70 / 100%. There’s no 90% level.
7. **Symptom checklist:** the VA rating-schedule symptoms, including suicidal ideation, neglect of hygiene, obsessional rituals, and persistent danger to self.
8. **Behavioral observations:** how you present on camera.
9. **Competency:** “Is the Veteran capable of managing his or her financial affairs?” See the note in section 5.
10. **Remarks:** including any test results.

The form also tells the examiner to stop the interview and get help if a mental-health emergency happens during it.

The MST marker opinion. VA rules let behavior changes after an assault (markers) back up an in-service assault that was never reported. Examples include substance abuse, depression or panic with no clear cause, changes in work performance, disregard for authority, and changes in relationships (38 CFR 3.304(f)(5); M21-1 VIII.iv.1.E.1.d via RateMyVSO).

- **Post-service changes can count** if they can reasonably be associated with the event.
- **Her opinion carries weight.** VA treats it as “significant probative evidence” (M21-1 VIII.iv.1.E.1.d), and it has to be “credible, unequivocal, and nonspeculative” (E.1.e).
- **Not reporting it in service can’t be held against you.** Per *AZ v. Shinseki*, an examiner can’t use the absence of an in-service report against you, and an opinion that does “should be returned as

insufficient" (M21-1 VIII.iv.1.C.2.a via Navigating VA).

- **What's in your file:** Reyes describes withdrawal and heavier drinking from summer 2012 and your disclosure to him in late 2013 (`REASSESSMENT-2026-10-08-CP-EXAM.md`, evening update). There's also the late-2012 drinking-related discipline (`COMPREHENSIVE-FINDINGS-2026-07.md` §2). Describe what actually changed after 2012; she connects the dots.

Afterward: the DBQ goes to a rater, who has to weigh the whole record, not just the exam (38 CFR 4.126(a)). Ask James to request a copy of the DBQ.

2A. How the interview usually runs

VA's examiner training for Initial PTSD exams says the examiner reviews your claims file **before** the exam, orients you, and then gathers history and symptoms. The DBQ is for writing up the report, "not meant as a guide for the examination itself" (VES examiner course, DMA Initial PTSD Examination). A typical sequence, with the time budget VA's older best-practice manual suggested (Best Practice Manual for PTSD C&P Exams, secondary copy):

1. **Orientation and consent.** She'll explain that she's an independent evaluator, not your therapist; the report goes to VA; confidentiality has limits; honesty matters and exaggeration hurts a claim; and the exam can be upsetting. Examiner guides recommend all of these (Worthen & Moering). She can't tell you results or answer questions about the claims process (VA.gov).
2. **Military history and the trauma** (~20 min). Your service, then the stressor.
3. **PTSD symptoms** (~40 min). Often in CAPS-5 style: for each symptom, **how often in the past month, how intense, since when, and how it affects you.**
4. **Mental status exam.** Mostly observation (see below).
5. **Psychosocial history and functioning.** Before, during and after service: family, marriage, work, education, legal, substance use, medications, treatment.
6. **Risk and closing.** Suicide/self-harm questions, possibly competency, then "anything else?"

The order varies by examiner. Some cover everything in 45 minutes; some take two hours or more.

The mental status exam (MSE) covers: appearance and behavior; speech; thought process and content (including obsessions, delusions, hallucinations); mood (what you report); affect (what she observes); and cognition (orientation, memory, concentration) (VES examiner course, above). On video she'll notice grooming, eye contact, restlessness, how you speak, and whether your emotion fits what you're describing.

- **Come as you normally are.** Don't clean up specially, and don't dress down to look worse. Either one makes the observation inaccurate.
- If you're having a better-than-usual day, say so and describe a typical one.

MST detail. The examiner has to document the stressor enough to assess it, and examiners are often asked to judge whether the evidence is consistent with MST. A 2026 National Academies report notes this often leads examiners to ask veterans to retell the event, and calls for a trauma-informed approach (NASEM, *Improving Disability Evaluations for Veterans Who Have Experienced MST*).

- Your 21-0781 and personal statement are in the file. If it's very hard to go into detail, you can say so and ask whether what's in your statement is enough. Then answer what she still needs.

- You can ask for a short break.
- Memory gaps from incapacitation are expected. Say what you remember and what you don't.

2B. Your rules and rights at the exam

Question	Answer	Source
Can I record it?	**No.** There's no right to record a VA exam, and refusing to be examined without recording can count as failing to report. Don't record, openly or secretly.	VA General Counsel Prec. 04-91
Can I have notes?	Yes, a short list: medications, key dates, and recent examples you want to be sure to mention. Use them as reminders, not a script to read. Bringing a record to the exam doesn't submit it to VA.	VA.gov; CCK Law
Can Erica sit in?	You can ask; the examiner may ask her to stay out. **Recommended: take it alone** so the examiner hears you unprompted. Erica stays reachable in case Dr. Howard asks to speak with her.	VA.gov
Can James (VSO) or a lawyer attend?	No right to counsel in the exam.	VA GC Prec. 04-91
Can I take a break, or stop if I'm in crisis?	You can ask for a break. The DBQ tells examiners to stop and get help in an emergency.	Initial PTSD DBQ
Will she tell me the result?	No.	VA.gov

How to describe symptoms so they're usable. Rating language turns on **frequency, severity and duration** (*Vazquez-Claudio v. Shinseki*, 713 F.3d 112 (Fed. Cir. 2013), as cited in `CLAIM-RECONCILIATION-WORKLIST.md` E1; not re-verified in this pass). For each symptom, have:

- **How often** in the past month ("4–5 nights a week," not "all the time" unless it literally is);
- **How bad** at its worst and on a typical day;
- **How long** an episode lasts and how long it's been this way;
- **One recent concrete example** with what it cost you (a missed appointment, a day in bed, leaving a meeting).

Estimates are fine; say "about" or "roughly." If you don't know, say so. Don't invent a number.

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3. Your record at a glance (for your reference, not a script)

When	What	Source
1989-05-30	Born	`COMPREHENSIVE-FINDINGS-2026-07.md` §2
~2007–09 (age ~18–19)	Kratom use begins (legal, store-bought; you didn't know it was an opioid). About 2–3 months with a Lake Charles psychiatrist for mild anxiety/depression around 19.	10/9 correction; 10/7 21-4138 Part 1 §2
2009-11-23	Enter active duty, USAF (F-15 crew chief, 4th Equipment Maintenance Squadron)	`COMPREHENSIVE-FINDINGS` §2; worklist B3a
May 2011	Left ankle fracture; morphine and oxycodone prescribed (your first opioid exposure as you recall it). **A 2015 VA intake says 2012.**	10/7 21-4138 Part 1 §1; worklist B2
2012	Harassment by a coworker's **civilian** wife, ending in the assault (summer / mid-to-late 2012). You were incapacitated and suspect you were drugged.	personal statement; 10/7 21-4138 §3; `CP-EXAM-PREP-MENTAL-HEALTH.md`
Late 2012	Drinking-related disciplinary action	`COMPREHENSIVE-FINDINGS` §2
2012–13	Heavier drinking and withdrawal (Reyes); Ambien/Restoril for insomnia	`REASSESSMENT` evening update; `COMPREHENSIVE-FINDINGS` §2
~mid/late 2012, in service	Poquette: met you in 2011; "after about 8–9 months" noticed a change, confronted you, and you told him	`supporting-docs/PocketBuddyLetter.md`; `COMPREHENSIVE-FINDINGS` §0.9
Late 2013, in service	Reyes: your disclosure "in late 2013, while we were both still serving." (Goff also, per your personal statement.)	`REASSESSMENT` evening update; personal statement
2013-11-22	Honorable discharge	`COMPREHENSIVE-FINDINGS` §2
Late 2013, after discharge	**Nick White** (high-school friend): you told him about the assault "toward the end of 2013 after he got out," and he "laughed it off." Roommates from 2015 during McNeese: isolation, declined invitations, disheveled, belongings disappearing, drug use framed as escape from "depression, anxiety, panic, and not being able to sleep."	Nick's corrected 21-10210 (`supporting-docs/VBA-21-10210-ARE-Nick-Complete-flattened.pdf`)
2013–15	Escalation to oxycodone/hydrocodone, then benzodiazepines	`COMPREHENSIVE-FINDINGS` §2
2015-09	VA Jennings CBOC: **"denies MST,"** negative PTSD screen	`COMPREHENSIVE-FINDINGS` §2
2016-07/08; ~2017-05	Acadiana Addiction Center, inpatient	10/7 21-4138 Part 1 §4
2016-12	B.S. Computer Science, McNeese	`COMPREHENSIVE-FINDINGS` §2

When	What	Source
2018-08 to 11	Victory: detox, residential, IOP. Intake lists opioid pills at 18 and cocaine at 19; trauma **"denies."**	10/7 21-4138 Part 1 §1; `COMPREHENSIVE-FINDINGS` §2
2020–24	Dr. Charles Bowers, psychiatrist (on and off; individual and couples)	10/7 21-4138 Part 2
2021	Disclose to Erica	`CP-EXAM-PREP-MENTAL-HEALTH.md`
2022-01-07 → 02-12	Starlite residential (DVT/PE complication). LEC-5 records an "unwanted sexual experience"; therapist Melissa Robbins. The same admission also has a "denies" trauma entry.	`COMPREHENSIVE-FINDINGS` §2; `REASSESSMENT` evening update
Nov 2022	Marry Erica Jure	`COMPREHENSIVE-FINDINGS` §2
2023-02-06 →	Cherie Mills, LCSW, outpatient	Cherie 07-17 letter
2023–25	Starlite stays per the records: 2023-01-08 → 01-22 (intake records age-18 history; trauma "None Reported"), 2023-07-20 → 08-17, 2024-07-30 → 08-13 (during Metal), 2024-12-26 → 2025-01-03. **Your signed Part 2 lists "January 2022, February 2023, August 2023, December 2024"** (see §4 item 23).	`COMPREHENSIVE-FINDINGS` §2; 10/7 21-4138 Part 2
2024	VA claim (not MST-based) denied: VA couldn't get Bowers's records	`CP-EXAM-PREP-MENTAL-HEALTH.md`; `COMPREHENSIVE-FINDINGS` §2
2025	Ocean's (Apr), Better Days (Sep); Reichmann telehealth meds from ~Jul	10/7 21-4138 Part 2
2026-04-17 → 05-11	FMLA leave from Mountain; Harmony residential (24 days by the dates); Zoe PHP after	10/7 21-4138 Part 2; worklist B3c
2026-05-19	Supplemental claim received	`COMPREHENSIVE-FINDINGS` §2
2026-06-16/17	Reyes statement; 21-0781 and personal statement signed; **full disclosure to Cherie on 6/17**	`COMPREHENSIVE-FINDINGS` §2; Cherie letter
2026-06-26 or 06-30	Mountain ends: agreement says 6/26, your 21-4138 says 6/30	`REASSESSMENT` open risks
2026-06-29; 07-12	Poquette statement; Erica statement	`COMPREHENSIVE-FINDINGS` §2; worklist B12
2026-10-13	Nick White lay statement (21-10210), corrected version: college with you 2007–2009; a short pre-service "period of depression" with possible therapy (matches your 10/7 account); late-2013 disclosure; house together 2015; post-service change. Signature on the corrected version was carried over from the original (re-signed copy not yet on file).	`supporting-docs/VBA-21-10210-ARE-Nick-Complete-flattened.pdf`; original: `VBA-21-10210-ARE-Nick-Signed-complete.pdf` (superseded)
2026-07-02; 07-17	Cherie nexus letters (two versions)	`REASSESSMENT` §4 (C6)

When	What	Source
2026-08-02	Metal manager statement	`supporting-docs/MetalManagerEmploymentStatement.pdf`
2026-08-04; 08-05	Two ER overdoses. Records: 8/4 gabapentin "accidental"; 8/5 **kratom**, "acute substance abuse"; "no SI." Your account: both were 7-OH at toxic doses, with SI you didn't disclose.	`CP-EXAM-PREP-MENTAL-HEALTH.md`; overdose 21-4138 draft
2026-08-08 → 08-18	Victory Addiction Recovery Center, residential, 10 days. You stopped using for a short time afterward. In your filed overdose 21-4138; no facility records in the file yet.	Your account (10/13); filed overdose 21-4138
2026-10-07	Two 21-4138s: pre-service history, perpetrator correction, full treatment history, Mountain end date	`supporting-docs/VA Form 21-4138 Part 1/2`
2026-10-09	Overdose 21-4138 uploaded. Kratom correction signed (upload status unconfirmed).	your report 10/9; `supporting-docs/VA_21-4138_Substance_Use_Correction_Flattened.pdf`

Jobs (`CP-EXAM-PREP-MENTAL-HEALTH.md` job table; Metal statement):

Employer	Dates	Leave	How it ended
Meta	3/28/2022 – 7/28/2023	FMLA (your account); Starlite Jan 2023	Company layoff (WARN notice)
DISQO	Jul 2023 – Mar 2024	FMLA (your account); Starlite from 7/20/2023	You know; not in the documents

⚠️ **The July 2023 dates overlap.** Starlite began 7/20/2023, before Meta's 7/28 end date, yet the old prep doc called the stay "weeks into" DISQO. Know the real sequence: was 7/28 a WARN-pay end date, and when did DISQO actually start? (§9.) | Metal | 4/15 – 12/6/2024 | Sick leave, schedule flexibility, reassigned work (7/18, 11/15, 11/21/2024) | Headcount reduction | | Mountain (MNTN) | May 2025 – Jun 2026 | Better Days Sep 2025; FMLA from 4/17/2026 | Involuntary termination with severance |

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4. Discrepancies the examiner may raise

The honest way through each is the same: say what's true, point to the correction if one exists, and say "I don't know" or "I don't remember" where that's the truth.

#	Item	What the records say	Correction status	How to handle it honestly
1	**Disclosure timeline** (C3)	Cherie's letter: first disclosure to her on 6/17/2026, four weeks after the 5/19 claim. Earlier disclosures: Poquette ~2012, Reyes late 2013, Erica 2021, Starlite LEC-5 2022, the 2024 claim.	No correction needed, but this is the **#1 credibility risk** (`REASSESSMENT` §5)	Walk the real sequence (section 3). Explain in your own words why you told some people and not others, including why you hadn't told Cherie in three years.
2	**Trauma denials**	2015 VA, 2018 Victory ×2, Starlite 2022–24: "denies" / "none reported"	n/a	Describe what was actually going on each time: avoidance, shame, not calling it assault. Note: *AZ v. Shinseki* covers not reporting it **in service**. It doesn't cover these later denials to clinicians, which VA can weigh for credibility. That's why an honest account of each one matters. Don't argue the law; describe your experience.
3	**Single incident vs. a year of harassment** (H1)	Letter: "single incident trauma." 21-0781: harassment through 2012.	None	Describe both: the harassment period and the assault.
4	**Perpetrator's status**	21-0781 Item 9: "female service member." Everything else: a coworker's civilian wife.	**Corrected**, 10/7 21-4138 §3	Give the accurate answer and say you filed a correction.
5	**Pre-service substance use**	Victory 2018: opioid pills at 18, cocaine at 19. Starlite Jan 2023: use from about 18. Personal statement: "minor alcohol and maybe a little marijuana."	**Corrected 10/9**: kratom from about 18, legal, you didn't know it was an opioid; no recalled cocaine. The "after 2024" error was corrected separately on 10/9.	Cover the facts in your correction: when the kratom started, that it was legal and store-bought, what you understood it to be, and that you've filed a correction. Cocaine: what you recall (none before service). Victory: your signed correction says you **believe its opioid entry reflects your kratom use**, plus the detox-day context. Know that the examiner has that sentence. Don't argue that kratom isn't an opioid.

#	Item	What the records say	Correction status	How to handle it honestly
6	**In-service opioids**	10/7 21-4138: morphine **and** oxycodone for the ankle. 10/9 correction: “other than the prescription I received for oxycodone.”	Minor wording gap	Give the full answer: both were prescribed for the fracture.
7	**August overdoses**	ER records: 8/4 gabapentin “accidental”; 8/5 kratom, “acute substance abuse”; “no SI,” “not a danger to himself”	**Corrected 10/9**, overdose 21-4138 uploaded	Tell the true version and own the gap: 7-OH, the amounts, your state of mind then, and why you understated it at the hospital. The 8/5 record already names kratom; what’s inaccurate is the gabapentin attribution and the “no SI.” Don’t try to match those parts.
8	**Sobriety in July**	Cherie’s 7/16 note: “history, currently sober.” Your account: heavy 7-OH use for about two years before the August overdoses.	**Open**	If asked, explain what was true in July (sober then, or not telling Cherie). Don’t guess. If you weren’t honest with her, it’s better said than discovered.
9	**SI history**	Seven denials, Apr–May 2026 (Naficy ×5, Harmony discharge, FMLA eval 5/14). “Every day when I wake up” (6/17). Cherie’s letter explains inpatient denials as avoiding involuntary holds.	The ~6/25 “denial” **does not exist** (retracted)	Describe SI as a **pattern over time**. Explain each denial by its real circumstances. The hold explanation applies only where it’s true; mid-May was a period without SI.

#	Item	What the records say	Correction status	How to handle it honestly
10	**Marriage**	Personal statement: “healthy and supportive.” Cherie: “total impairment… caregiving dependency.” Erica: separation, considered divorce.	None	Describe it as it is now. If support and strain are both true, say both.
11	**ADLs**	Erica and you: she prompts and manages. Cherie: during bad stretches she “takes over.” Naficy: “well-groomed” at five inpatient visits.	None	Describe exactly what Erica prompts, how often, and what gets skipped without her. **Prompting is not doing it for you.** Don’t say “feeds me” or “cuts my hair.”
12	**Delusions**	Cherie: “paranoid delusions” (the fear of losing your job). Naficy: “denies delusions.”	None	Describe the persecutory worry as a recurring worry: what it is, how often, how convinced you are, whether you can step back from it. Don’t use the word “delusion.” You describe the “grandiose” belief as ordinary self-belief; classifying it is Cherie’s and the examiner’s call. When the worry started is unsettled in your file (after service per 10/8, about 18 years ago per a 07-06 note); give the true answer. **The job-loss fear is one case of a longer pattern** (§5, “Certainty you’ll fail”): at McNeese after service you were sure you’d fail tests and got A’s; at work you’re sure each project will fail; with people you’re sure they won’t like you. Describing the pattern explains what Cherie wrote better than either label does.
13	**Family and social contact**	Cherie: estranged from family for 6–7 years; speaks to no one else.	Your 10/8 clarification: strained, little contact	Be concrete about how often, who reaches out, and what contact is like. Your 10/8 account is little contact, strained when it happens. Don’t say total estrangement unless it is.
14	**Layoffs vs. terminations; “FMLA at every job”**	Meta and Metal were layoffs; Mountain was termination after FMLA. Cherie: “FMLA… at every job.”	None	Call the layoffs layoffs, then describe attendance, leave and accommodations at each job. Use “medical leave” unless a document says FMLA. Metal was sick leave and accommodations.
15	**Mountain end date**	Agreement: 6/26. Your 21-4138: 6/30.	Open (minor)	Use the date you can stand behind; it’s fine to point to the agreement’s date.

#	Item	What the records say	Correction status	How to handle it honestly
16	**Harmony length**	21-4138: “25 days.” The dates work out to 24.	Open (minor)	If asked, give the approximate length; the discharge dates are the reliable source.
17	**Age at the incident**	CPT worksheet: “twenty-two.” DOB 5/30/1989; the assault was summer or later 2012 (you’d be 23).	Open (minor)	Give the age that matches the month it happened.
18	**Ankle date**	Your statements: May 2011. 2015 VA intake: 2012.	Open	Give the date you’re confident of.
19	**Gaming**	Erica: frequent all-night gaming. Jan 2023 Starlite: stopped “some years ago.”	n/a	Both can be true at different times. Answer for the period asked about.
20	**Two “Recover” PsychEvals** (5/14, 5/15)	Different client IDs. The 5/15 one attributes PTSD to “childhood trauma and military experiences.”	Open; don’t submit either	If asked, say what you know. If you know of no childhood trauma, say so plainly.
21	**Amphetamine SUD code** (F15.20, Harmony)	Coded at discharge, unexplained	Open	If you don’t know what drove the code, say so.
22	**Two nexus letters** (C6)	7/2 and 7/17 versions may both be in the file	Ask James	Nothing for you to say. Know that she may compare them.
23	**Starlite dates in your signed Part 2**	Part 2: “January 2022, February 2023, August 2023, December 2024.” Records: Jan 8–22, 2023 (not February), and a **Jul 30–Aug 13, 2024** stay that Part 2 leaves out.	Open	Know the real dates (§3). If asked, give the correct ones and say the form was from memory.

#	Item	What the records say	Correction status	How to handle it honestly
24	**Pre-service prescription**	A 07-06 note records you describing a separate pre-service medication you didn't know was an opioid. Your signed filings mention only kratom and the 2011 ankle prescriptions.	Open	If asked about prescriptions before 2009, say what's true. Don't guess.

5. Likely questions by area

For each area: what she's assessing, the facts from your file, and what to make sure comes across. Use your own words.

History before service (DBQ history section)

- **What she's assessing:** whether a condition existed before service and service made it worse, or started in service.
- **The facts:** kratom from about 18; 2–3 months of outpatient care around 19 for mild anxiety/depression (10/7 21-4138 §2); no recalled cocaine.
- **Make sure it comes across:** what life looked like before 2009 versus after 2012, concretely: work, drinking, sleep, relationships. The change after 2012 is the point.

The stressor

- **What she's assessing:** Criterion A, plus the marker opinion.
- **The facts:** harassment through 2012; the assault; incapacitation with a suspected drugging (describe it as a suspicion); memory gaps.
- **Make sure it comes across:** what you remember and what you don't. Memory gaps from incapacitation are expected, so don't fill them in. Then describe what changed afterward in service: drinking, withdrawal, sleep, discipline, telling Reyes.

PTSD symptoms, Criteria B–E

- **What she's assessing:** frequency, severity and duration.
- **Make sure it comes across, for each:** how often, how bad, a real example.
    - nightmares and intrusive memories;
    - avoidance of reminders and of women (10/7 21-4138 §2);
    - **women at gatherings** (your account, 10/13): you feel on guard around women at gatherings, expect one of them to harm you somehow, and move away from them. This is avoidance and hypervigilance tied to the stressor by what it's about. Describe what you do (where you stand or

sit, leaving early, skipping events), what you expect could happen, how sure you are in the moment, how often, and a recent example. Leaving Zoe PHP early over coed housing and leaving a crowded 12-step meeting may be examples, if they're true ones. Describe the behavior and the fear instead of using the label "paranoid."
    - beliefs about yourself and the world;
    - startle, hypervigilance, concentration, irritability;
    - sexual dysfunction, if you're willing to discuss it. It's in Cherie's letter and Erica's statement.

Criterion H and SUD (DBQ items H, 3B and 4B)

- **What she's assessing:** whether symptoms come from substances, and whether impairment can be separated by diagnosis.
- **The facts:** Cherie's opinion that SUD is at least as likely as not secondary to PTSD; heavier drinking from 2012 and opioid escalation after discharge; the 7-OH.
- **Make sure it comes across:** how use relates to your symptoms as you actually experience it: when you use, what you're escaping, what happens to anxiety, panic and sleep. Be honest about **current** use: what, how much, last use, treatment. If you can't tell which symptoms come from which condition, saying so is legitimate. (If symptoms can't be separated, VA attributes them all to the service-connected condition: *Mittleider v. West*, 11 Vet. App. 181. You don't need to argue this.)

Depression, anxiety, OCD

- **The facts:** panic "more mornings than not" (Cherie); DASS-21 extremely severe on all three scales; WHODAS about 71% (Cherie's letter).
- **Make sure it comes across:** the real frequency of panic attacks, and how much time obsessions and rituals take each day. Sleep, appetite and energy. You had mono in July (7/16 note); it explains some of the fatigue then, so separate it out honestly.

Certainty you'll fail or be disliked, despite the evidence (your account, 10/13)

- **What she's assessing:** thought content (part of the mental status exam), negative beliefs about yourself (PTSD Criterion D2 covers "persistent and exaggerated negative beliefs or expectations about oneself"), and worry. Which diagnosis it belongs to is her call, not yours.
- **The facts:**
    - **School:** at McNeese (2013–2016, after service) you were convinced before tests that you'd fail, and you got A's.
    - **Work:** you're convinced each project will fail, and the projects have worked out. Cherie's "paranoid delusions" (the fear of losing your job) is the same pattern applied to your job.
    - **People:** you're convinced people won't like you, and they usually do.
- **Make sure it comes across:**
    - how sure you feel while you're in it, and whether past results talk you out of it (they don't);
    - how often it happens and how long it lasts: before each test? each project? every time you meet someone?
    - what it costs you: sleep, panic, overworking and needing control (ties to the OCD item), dread before deadlines, pulling back from people;
    - when it started: you place it after service. If you remember anything like it before 2009, say so.
- **Two cautions:**

    - **Don’t call it “delusions.”** That’s a term for psychotic symptoms, and it contradicts Naficy’s “denies delusions.” Say “I’m convinced I’ll fail, even though I know my track record says otherwise.”
    - **Don’t say “every project succeeded” if Mountain is on the table.** Your file has Mountain ending involuntarily, with a negotiated severance, and the remediation plan describes it as a performance/PIP loss. Keep the two apart: the fear runs even when the work goes well, and separately, the last job ended the way it did. Successes said with no context also read as strong functioning, so add what the fear costs you.

Suicide risk

- **What she’s assessing:** current risk, history and protective factors (the DBQ symptom checklist includes SI and “persistent danger of hurting self”).
- **Make sure it comes across:**
    - the pattern over time;
    - what was true at the August overdoses;
    - what’s true **today**: thoughts, plans, access to means, what keeps you safe.
- If you’re asked about something on the day, answer truthfully. She’s required to stop and get help if there’s an emergency, and that’s a safety measure, not a penalty.

Work

- **What she’s assessing:** reliability, productivity and attendance (the 70% language is “deficiencies in most areas”).
- **Make sure it comes across:** walk the job table in order. For each job: what led to each leave, what coming back was like, how it ended. The core point: you can get jobs, but you can’t keep working steadily. You haven’t worked since June 2026.

Social life, marriage, ADLs: see items 10–13 in section 4. Describe a **typical** day and a **bad** day.

Competency (DBQ section 9 above)

- **What she’s assessing:** whether you can manage your own money.
- **Why it’s relevant here:** your heavy spending on 7-OH bears directly on this question. Your 10/8 account gives more than $30,000 over about two years, and about $500 a day at the time of the overdoses. Those two figures don’t reconcile over two years, so know which is true and for what period (§9).
- **Answer honestly**, including who actually handles the money now.
- **What a “no” means:** the examiner’s answer is one piece of evidence. The rating agency decides competency from the whole record and resolves doubt in favor of competency. If it proposes an incompetency rating, you get notice and a chance for a hearing first. A fiduciary is appointed only after a final rating (38 CFR 3.353; 38 CFR 13.100). An incompetency rating can have other consequences, such as firearm background-check reporting (not verified here). Ask James before the exam if this worries you.

Before the call ends

- Say anything important that wasn’t covered.

- If you're holding it together on camera, it's fair to say that isn't your average day.

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6. Testing, and why careful answers protect you

- **What she may use:** checklists such as the PCL-5 (20 items, 5–10 minutes, not a stand-alone diagnosis), a structured interview such as the CAPS-5 (45–60 minutes), and possibly a longer inventory with **validity scales** (MMPI-2/2-RF/3 or PAI) or a short screener such as the M-FAST or SIMS (VA/DoD PTSD assessment guidance; National Center for PTSD, PCL-5; Worthen & Moering, practical guide for C&P examiners).
- **Many exams use only an interview.** VA research found structured instruments are rarely used in C&P exams (VA HSR&D Forum, 2012). Don't be surprised either way.
- **What validity scales do:** they flag answers that endorse unusually many, or unusual, symptoms. Examiners are taught to separate a "cry for help" pattern from extreme distress versus actual exaggeration. They report results in Remarks, and a clearly invalid profile can lead an examiner to defer or withhold a diagnosis (Worthen & Moering). The National Academies note these scales can't, on their own, prove malingering (NAP, 2007). In practice, though, an invalid result undercuts the whole exam.
- **What this means for you:** read each item, answer for the time period it asks about, and pick the answer that's true, not the strongest one. Rate symptoms you don't have as absent. Accurate answers are what make your real symptoms count.

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7. Exam-day setup

- **Where:** in Texas, alone, in a private room with the door closed. Erica out of the room and out of earshot. If Dr. Howard wants to hear from Erica, she'll ask, and Erica answers in her own words.
- **When:** join by 11:50. If no one has joined after 15 minutes, call VES (877-637-8387).
- **Have with you:** photo ID and your medication list.
- **Be sober.** Don't take anything that isn't prescribed that morning.
- **Have a backup:** your phone charged as a hotspot, the VES number written down.
- **If you get overwhelmed during the exam:** say so and ask for a minute. That's allowed.

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8. After the exam

1. **Right away, write notes.** Date and time them. Record the start and end time, roughly what was asked, what you answered, whether she referred to your records or any testing, anything you didn't get to say, and anything that seemed wrong. Then debrief with someone. Notes taken right after the exam are the best evidence if the report later gets things wrong (claim.vet, secondary).
2. **If something important wasn't covered:** send VA a short 21-4138 through James saying what you didn't get to describe. Facts only.
3. **Get the DBQ.** James may be able to see it in your claims file. Otherwise request it on **VA Form 20-10206** at VA.gov under "claim exams," with the exam date. Asking only for the completed DBQs is faster than requesting the whole claims file (VA Claim Commander; RateMyVSO guide). Check it line by line against §3.

4. **If the report has errors and there's no decision yet:** send a 21-4138 through James listing each specific error ("the report says X; the record at [date] says Y"). Keep it calm and factual, add contradicting evidence, and ask for a new exam if it's warranted. VA decides whether to order one (VA Claim Commander).
5. **Choose a lane by outcome** (`REASSESSMENT-2026-10-08-CP-EXAM.md` §3):
    - **TDIU timing, ask James now:** whether to file the 21-8940 **now**, so TDIU is decided with the initial rating (the record already raises unemployability, *Rice v. Shinseki*), or right after a grant. Check the VR&E interaction first (`vre/OPTIONS-REPORT-2026-09-14.md` §7).
    - **Favorable:** service connection at 70% or 100%.
    - **Mixed** (diagnosis but no MST link, or 50%): Higher-Level Review if the opinion ignored the markers or gave no reasoning; otherwise a supplemental claim with a psychologist or psychiatrist opinion.
    - **Unfavorable:** the same options, plus a Board appeal with a hearing. One year from the decision.
6. **Sleep apnea:** file the Intent to File (21-0966) now if it isn't already filed. At 70% mental health it gets you to 90% combined, not 100% (`REASSESSMENT` §1).
7. **Hypertension:** lisinopril since 2021 makes it a possible secondary claim (`REASSESSMENT` late update). Ask James.
8. **Update the worklist** (`CLAIM-RECONCILIATION-WORKLIST.md` I-8) once the kratom correction is confirmed received.

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9. Unresolved: ask me or James

1. ~~The "after 2024" line~~: corrected 10/9. Confirm with James that the kratom correction and the fix both show as received.
2. **What the uploaded overdose 21-4138 actually says.** This doc relies on the 10/8 draft (`personal-records/21-4138-AUGUST-2026-OVERDOSES-DRAFT.md`). If the final text differs, the final text controls.
3. **Which nexus letter(s) VA holds** (7/2, 7/17, or both).
4. **Cherie's 7/16 "currently sober"** vs. two years of 7-OH use. What was true in July?
5. **Mountain end date:** 6/26 or 6/30.
6. **The Goff statement** promised in the 21-0781. Has VA been told it can't be obtained? (worklist B11b)
7. **The two Recover PsychEvals** and the "childhood trauma" phrase (worklist I-17).
8. **Nexus letter provenance:** did Cherie approve the final text and the image signature? (`REASSESSMENT` §5)
9. **The month of the assault** (affects 22 vs. 23).
10. **Starlite dates in Part 2** ("February 2023"; the missing Jul–Aug 2024 stay). Does it need a clarifying 21-4138?
11. **July 2023:** Meta's real end date vs. DISQO's start.
12. **7-OH spending:** more than $30,000 over two years vs. about $500 a day. Which is true, for what period, and what does the uploaded overdose 21-4138 say?
13. **When the persecutory worry began** (after service vs. ~18 years ago), and whether you've talked it through with Cherie. (Settled 10/13: the certainty-of-failing pattern began at McNeese, after service.)
14. **The pre-service prescription** mentioned in a 07-06 note (§4 item 24).

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10. Sources

Corpus: listed in the status header and inline. Key signed filings:

- `supporting-docs/VA Form 21-4138 Part 1 Official (1).pdf` and `Part 2` (10/7)
- `supporting-docs/VA_21-4138_Substance_Use_Correction_Flattened.pdf` (10/9)
- `supporting-docs/Marcus Klein Cherie Mills Nexus Letter -2026 (3).pdf` (07-17)
- `supporting-docs/Cherie CPT Progress Note.pdf` (7/16)
- `supporting-docs/MetalManagerEmploymentStatement.pdf` (8/2)

External (research notes in the session scratchpad):

Source	Used for	Status
VA.gov, VA claim exam	Length, rescheduling, examiner gender, missed exams	Primary (VA)
VES FAQ	5-day reschedule window	Primary (contractor)
FedLaws, contractor exams (2026-09-16)	Telehealth video and in-state licensure rule	Secondary, cites VA PWS
Initial PTSD DBQ	DBQ sections	Internal VA form via third-party mirror; version unconfirmed
PTSD Review DBQ	Impairment-level wording	Primary (VA)
M21-1 VIII.iv.1.E and 1.C via RateMyVSO, Navigating VA	Markers, weight of the marker opinion, *AZ* rule	Mirrors of VA manual
38 CFR 3.304(f)(5); 3.655(b); 4.126(a); 4.130	Markers; missed exam; whole record; rating levels	See citation check below

Source	Used for	Status
AZ v. Shinseki, 731 F.3d 1303; *Mittleider v. West*, 11 Vet. App. 181; *Rice v. Shinseki*, 22 Vet. App. 447	Non-reporting; inseparable symptoms; TDIU	See citation check below
VA/DoD PTSD assessment; NCPTSD PCL-5	Instruments	Primary (VA)
HSR&D Forum 2012	Structured tools rarely used	Primary (VA research)
Worthen & Moering, *Psychological Injury and Law*	Validity screening practice	Peer-reviewed
NAP 2007, PTSD C&P ch. 4	Limits of validity scales	National Academies
VES examiner course: DMA Initial PTSD Examination	File review, exam sequence, MSE areas	Contractor training (2019)
VA GC Prec. 04-91	No right to record or to counsel	Primary (VA OGC)
NASEM 2026, MST disability evaluations	Retelling MST; trauma-informed exams	National Academies
VA Form 20-10206; VA Claim Commander; RateMyVSO DBQ guide; claim.vet; CCK Law	Getting and challenging the report; notes	Primary form plus secondary guides
H.R. 3951 (engrossed); S. 3703	Cross-state licensure	Not enacted per govinfo bill status (records through 2026-06-25)

Consistency check (2026-10-09): `claim-consistency-checker` found 3 HIGH and 8 MEDIUM items. All are fixed above or listed in §9. It couldn't locate Erica's signed statement or the uploaded overdose 21-4138, so it didn't check claims about them.

Citation check (`citation-verifier`, 2026-10-09):

Citation	Result
38 CFR 3.304(f)(5)	Confirmed. “Disregard for authority” and “relationship changes” come from the M21-1 list, not the regulation.
38 CFR 3.655(b)	Confirmed for supplemental claims. Conditions added inline.
38 CFR 4.126(a); 4.130	Confirmed
AZ v. Shinseki, 731 F.3d 1303	Confirmed for non-reporting in service. Removed where it had been applied to post-service denials (§4 item 2).
Mittleider v. West, 11 Vet. App. 181, 182	Confirmed (primary text not reached; via BVA decisions)
Rice v. Shinseki, 22 Vet. App. 447, 453–54	Confirmed. Whether this record raises TDIU is an application, not Rice’s holding.
M21-1 VIII.iv.1.E.1.d/e, C.2.a	Substance confirmed. Topic numbers corrected (mirrors; change date 2025-09-16).
Competency / fiduciary	Corrected to the 38 CFR 3.353 / 13.100 sequence
H.R. 3951 / S. 3703 / S. 1533	Not enacted (govinfo bill status)